

Thymosin Beta-4 (TB-4)

Thymic peptide · 2.5 mg/mL injectable · Tissue repair / immune modulation

SubQ Injection

Twice Weekly

2.5 mg/mL

Investigational

COMMON INDICATIONS (INVESTIGATIONAL)

- ▶ Soft tissue injury
- ▶ Tendon / ligament repair
- ▶ Muscle recovery
- ▶ Wound healing
- ▶ Chronic inflammation
- ▶ Hair loss / alopecia (topical)
- ▶ Immune modulation
- ▶ Long COVID / chronic fatigue
- ▶ Athletic recovery

STANDARD DOSING PROTOCOL (2.5 MG/ML)

LOADING

5 mg/wk

Split: 2.5 mg 2× wk

STANDARD

2.5 mg

2× weekly SubQ

MAINTENANCE

2 mg

Weekly SubQ

CYCLE

4–6 wks

Then reassess

DOSING NOTES

Most common clinical protocol: 2.5 mg twice weekly for 4–6 weeks, then reassess. Loading phase of 5 mg/week (split into 2 injections) may be used for acute injury. Inject SubQ in abdomen or near injury site. TB-4 has a long half-life and slow onset — expect 2–4 weeks before noticeable response. Investigational; reserved for short courses with defined endpoints. Often paired with PDA/BPC-157 for synergistic tissue repair.

VOLUME REFERENCE (2.5 MG/ML = 2,500 MCG/ML)

TARGET DOSE	VOLUME TO DRAW	U-100 SYRINGE	NOTES
1.25 mg	0.50 mL	50 units	Lower-dose protocols
2.0 mg	0.80 mL	80 units	Maintenance weekly
2.5 mg	1.0 mL	100 units	Standard dose, 2× weekly
5.0 mg (loading)	2 × 1.0 mL	2 × 100 u	Split across 2 injections in same week

STORAGE & COUNSELING

STORAGE

- ▶ Refrigerate at 36–46°F (2–8°C)
- ▶ Do not freeze
- ▶ Protect from light
- ▶ BUD per pharmacy label after first puncture

CAUTIONS & COUNSELING

- ▶ Avoid in active malignancy (pro-angiogenic / cell migration concern)
- ▶ Limited human safety data — investigational
- ▶ Pregnancy/lactation — not recommended
- ▶ Often combined with PDA for soft tissue protocols